



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نجلاء احمد على محمد
المحافظة : القاهرة
الحى / المدينة : حلوان
العنوان : 000
رقم البطاقة : 27802112102087 رقم التليفون : 01028041123
العمر (تاريخ الميلاد) : 11-02-1978 المهنة : بدون عمل
تاريخ تسجيل الإستمارة : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه
[11112]
قسم العلاج :
الأورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات بالمرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نجلاء احمد على محمد National ID: 27802112102087	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



05/24/2026

MDT FORM

Name:	Najlaa Mohamoud Ali Mohamoud	MRN:	6464
Age:	48 Y	ID:	27802112102087
Menopausal Sta	post	Phone:	01028041123
NRS SCORE:		Marital Status:	married
BMI:		Family History:	-ve
CO Morbidity:	free	Reason Of MDT:	post op

Clinical Data:

BX: IDC G111
ER: 7/8 **PR:** 7/8 **HER2:** 3+ **KI67:** 80-90%

R**L****Sonomammography:**

T4b N3a M

Mets Workup:

CT : free

bone scan equivocal tracer uptake at lt stercoclavicular region and lt 6th rib for further radiological correlation and follow up

History:

Initially: T4b N3a M??

CT : free

bone scan equivocal tracer uptake at lt stercoclavicular region and lt 6th rib for further radiological correlation and follow up

pt received NACT

TXT + Carbo 6 cycles

and 2 cycles of physgo

Lt MRM was done

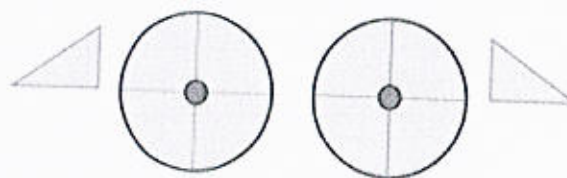
free margins

LN 0/11

Recommendations:**MDT Outcome & Plan**

for adj PHESGO

PORT

FEMARA 7 years And E2 Level Fsh Lh to confirm menopausal status
zometa every 6 ms**T N M****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr Hanan Ezzat					

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نجلأ
احمد على محمد

٦ ش عمر بن الخطاب - ترسا
مركز أبو النمرس - الجيزة

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ربة منزل

أنثى مسلمة متزوجة

محمد عبدالمنعم محمد الراجحي

البطاقة سارية حتى ٢٠٣٢/٠٥/١١

١

03/01/2026

MDT FORM

Name:	Najlaa Mohamoud Ali Mohamoud	MRN:	6464
Age:	48 Y	ID:	
Menopausal Sta	post	Phone:	01028041123
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	free	Reason Of MDT:	post chemo

Clinical Data:

BX: IDC GIII
ER: 7/8 **PR:** 7/8 **HER2:** 3+ **KI67:** 80-90%

R

L

Sonomammography:

Mets Workup:

History:

Initially: T4b N3a M??
 CT : free
 bone scan equevocal tracer uptake at lt stercoclavicular region and lt 6th rib
 for further radiological correlation and follow up
 pt received NACT
 TXT + Carbo 6 cycles
 and 2 cycles of physgo ???

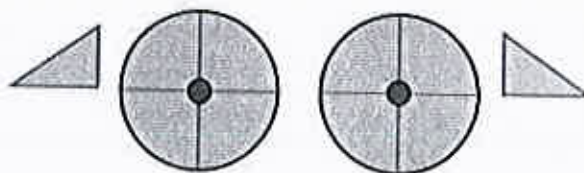
surgical review:

clinically: still residual skin edema
 pt is candidate for lt MRM
 omnia essawy 15/2/26

Recommendations:

MDT Outcome & Plan

lt MRM



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نورهان مصطفى	د/ احمد نجيب				

Patient Name: Naglaa Ahmed Ali Mohamed	Date : 07/12/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 47 Year(s)
Medical ID:	Pat. ID: 132996

MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

Reported case of left breast cancer

I- CHEST:

- Left breast speculated mass LOQ measures 5 x 2.8 cm with overlying skin thickening .
- Left enlarged axillary lymph nodes , the largest measures 14x 8 mm
- Few tiny calcified bilateral scattered pulmonary nodules noted average 3 mm .
- No pulmonary masses or cavitary lesions.
- No gross hilar or mediastinal masses or lymph node enlargement.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion on non contrast basis or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Normal plain C.T appearance of the spleen with small splenule , pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no plain CT evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- IUCD in place . .
- No ascites.
- No worrisome lytic or sclerotic bony lesions .
- No vertebral structural collapse or wedging .

OPINION:

- No pulmonary , hepatic or bony metastatic deposits

Dr/Reham Khedr,,,MD

Name : NagalaAhmad

Date : 21-07-2025 03:13

Branch : Maadi

Medical No. : 157015

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY**History:**

- A 47-year-old lady, with history of left breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Enhanced tracer uptake is noted at the left sternoclavicular region as well as left 6th rib.
- No other abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Equivocal tracer uptake at the left sternoclavicular region and left 6th rib, for further radiological correlation and follow up.

*Much obliged**Dr. Mohammed AtaAllah, MSc
Nuclear Medicine**Dr. Alaa Nagi, MD
Consultant of nuclear medicine*



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

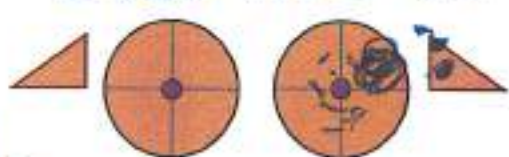
الست المصرية
صحة
مصر

MDT FORM

18/6/2022 G

Name	نجلاء - امير	MRN	10115-00
		ID	1501150
Age	47 y	Phone	1-99-81153
Menopausal status	post	Marital status	1-977722-
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	-ve		CVA-C11C10C-NV

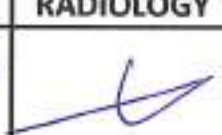
Clinical Data

BX:	IOC 2 III	R	Left B, G	L						
ER:	7/8	PR:	7/8	HER 2:	+++	Ki67:	90%	CTab	N3a	Mo
Sonomammography:										
- UOQ mass + skin thickening - Multiple A-W lobules I, II, III										
Mets Workup:								TNM		
CT CAP: No Mets										
Bone sc. pending										

Recommendations: for MDT, consider TCHP therapy
لدى

MDT OUTCOME & PLAN for. echo, lab
Needs TCHP x 8 then
MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					



Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة



PATHOLOGY REPORT

Name: نجلاء احمد على محمد

Patient's I.D :27802112102087

Age: 47 Y.

Clinical data: Left breast mass (BIRADS 5).

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

Pathology Number: 113/2025

Specimen Received Data: 27 / 04 /2025

Date Report Issued: 07/ 05 /2025

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of malignant tumor formed of groups & cords of markedly anaplastic ductal epithelial cells. Surrounding desmoplastic stroma with 5-10% TILs was seen.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone, Her-2/neu AND Ki-67 (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana IView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed moderate positive nuclear staining in most of tumor cells by chromogen.

Examination of section treated for progesterone receptors revealed strong positive nuclear staining in 40-50% of tumor cells by chromogen.

Examination of section treated for Her-2/neu revealed strong complete membrane staining in >10% of tumor cells by chromogen (score 3+).

Examination of section treated for Ki67 revealed positive nuclear stain in average of ±80-90% of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade III.
- **Positive** For Estrogen Receptors, Score 7/8.
- **Positive** For Progesterone Receptors, Score 7/8.
- **Positive** For Her-2/neu Over-Expression (Score 3+).
- **80-90%** Ki67 Proliferative index.

Prof. Dr. Passant Essam
Dr. Sara Abou-Fandoud

Passant
Sara

113/2025

ن. العبد



Name: naglaa ahmed ali	Date: 4/27/2025
Age:	ID: 27802112102087

Further evaluation by contrast enhanced mammography is recommended for better evaluation of the lesion extent.

- *Normal sonomammography study of right breast (BIRADS 1).*

Thanks for referral,

Dr.Sherihan Fakhry, MD

Dr. Nour El Houda Mohamed, MSc

Name : NagalaAhmad

Date : 21-07-2025 03:13

Branch : Maadi

Medical No. : 157015

Ref.Physician Prof : Gostaf Roseh Hospital

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Impression:

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Much obliged
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Dr. Alaa Nagi, MD
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مستشفيات جامعة القاهرة



PATHOLOGY REPORT

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Examination of section treated for Her-2/neu revealed strong complete membrane staining in >10% of tumor cells by chromogen (score 3+).

Examination of section treated for Ki67 revealed positive nuclear stain in average of $\pm$ 80-90% of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade III.
- **Positive** For Estrogen Receptors, Score 7/8.
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- **80-90%** Ki67 Proliferative index.

Prof. Dr. Passant Essam

Dr. Sara Abou-Fandoud

Passant
Sara



0696774840

Name: naglaa ahmed ali	Date: 4/27/2025
Age:	ID: 27802112102087

**Full Field Digital Mammography With Complementary
High Resolution Ultrasound Examination**

Indication:

- Left breast lump.

Findings:

- Scattered fibroglandular densities (ACR b).
- Left UOQ irregular speculated high density mass is noted measuring about (7 x 4.8) cm (based on CC view) with long spicules seen extending toward the nipple/areolar complex as well as to the overlying thickened skin with consequent dimpling.
- No clustered microcalcifications seen.

Complementary US:

- Corresponding left breast UOQ 2 O'clock irregular speculated mass with extensive desmoplastic reaction is noted, being at 0.3 cm from the overlying skin. Associated diffuse skin and areolar thickening along with interstitial edema is noted.
- No right breast solid or cystic lesions
- Normal skin thickening of the right breast.
- Normal right nipple /areola complex.
- Multiple pathological enlarged axillary LNS (level I,II,III,) are noted showing marked diffuse cortical thickening and eccentric fatty hila, yet one of them appears irregular denoting extracapsular invasion
- Right nonspecific axillary lymph nodes with preserved fatty hilum.

Opinion :-

- Left breast locally advanced malignant mass associated with diffuse edematous changes and ipsilateral pathologically enlarged level I, II, III axillary lymph nodes (BIRADS 5, for histopathological correlation.



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113/2025

Dr. Sara Abou-Fandoud